

So you're thinking about nursing

The short version — written for you, not about you.

Everything that matters, none of the bits written for your parents.

We scored twenty-seven careers on how exposed they are to AI. Nursing came out at **2.8 out of 10**, where 10 means most at risk.

An entry-level software developer scored 8.1.

So the assumption that tech is the safe bet and caring work is the soft option is, on the evidence, backwards.

The 60-second version

- Nursing is one of the safest careers we scored. That part is settled.
 - The protection is in the bedside, not the badge. Desk-based nursing jobs score nearly double.
 - AI is mostly taking the paperwork — which is the part nurses hate.
 - The real risk to a nursing career isn't AI. It's leaving.
 - There's one skill nobody is teaching yet, and you'd be early to it.
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Why it's protected

Four reasons, and none of them depend on how clever AI gets.

You can't turn a patient through a screen. Robotics is decades away from unstructured, hands-on work with a human body — not product cycles away.

People want a human present when they're frightened. That's not a preference anyone delegates to software.

The law requires a licensed one — licensure, scope of practice, staffing ratios. And law moves at the speed of politics, not technology.

Patients don't match the textbook. They present oddly, deteriorate unpredictably, and arrive with several things wrong at once. That's exactly where AI is weakest.

The bit nobody tells you

Same degree. Same licence. Very different exposure depending on where you end up.

WHERE YOU WORK	SCORE
Specialist clinical (ICU, ED, theatre)	2.4
Community / home health	2.7
Bedside RN	2.8
Nurse practitioner	3.2
Telehealth triage	5.2
Case management (desk)	5.4

Roughly double the exposure, for the same qualification.

Desk-based nursing jobs often come with better hours and less physical strain, so plenty of nurses drift that way over time. That can be exactly the right call for your life. Just make it on purpose rather than by accident.

What AI is actually doing to the job

Mostly taking the charting. Documentation, handover notes, care plans.

Here's the thing worth understanding, because it applies to any career you might consider: **AI goes after the routine, repeatable part of a job first.** That's true everywhere. What changes is whether the repeatable part was the bit people liked.

For a graphic designer, the repeatable part was the craft. So they end up checking a machine's version of their own work.

In nursing the repeatable part is paperwork — the thing nurses consistently say drives them out. Automating it gives time back to patients.

That's an unusually good deal, and it's worth knowing it isn't the norm.

The skill nobody is teaching

Knowing when the AI is wrong.

When an auto-generated note doesn't match the patient. When an alert is confidently, plausibly incorrect. **That's a clinical skill, not a technical one** — you can only catch a convincing error if you know the patient well enough.

Which means the boring foundational years matter *more*, not less.

And here's your opening. AI is already in most hospitals. It's arriving faster than the training to use it, and most working nurses are figuring it out alone. Only around 40% of nursing programmes have updated their curriculum for it.

So you'd arrive as **part of the first group who never had to unlearn anything**. In a profession where getting ahead normally takes years of experience, this is one dimension where being new is an advantage.

It won't make you better than a nurse with twenty years on a ward. It'll make you useful faster, and visible to the people who run projects.

One caution, because school will have told you the opposite. Using AI to skip the work leaves you weaker — and in nursing that eventually becomes a patient-safety problem rather than a grades problem. But *understanding* AI is a completely different thing. That isn't cheating and never was.

The part we won't soften

The biggest risk to a nursing career isn't AI. It's leaving.

The national survey covering 527 hospitals found **22.7% of new nurses leave within their first year, and 56.8% of everyone who quits had been there under two years.**

Though there's a twist worth knowing. Retention in those first two years is actually *good* — residency programmes hold people in place. **Turnover spikes at around thirty months, when the support comes off.** Which makes the quality of your first employer's graduate programme, and what happens when it ends, a much bigger deal than it sounds.

The reasons people leave are consistent: staffing pressure, shift work over decades, the physical toll, and not being able to give the care you know someone needs.

That last one has a name — moral injury — and it's different from being tired. It's what makes people walk away from a job they still love.

Two things worth knowing anyway.

Conditions measurably improved between 2022 and 2025 before slipping again this year — satisfaction went from 28% in 2022 up to 55%, then back to 47%. So this isn't a one-way decline, it's a recovery that stalled.

And satisfaction varies more *inside nursing* than between nursing and other careers — nurse educators report around 68% satisfaction, progressive care nurses 23%. Same profession, 45-point gap.

Here's the statistic that should actually shape your decision. In a survey of 7,723 nurses, 73% said they'd choose nursing again — but **only 45% would choose the same specialty.**

So the real question isn't "should I be a nurse." It's **what kind, and where.**

Does this actually sound like you?

It tends to suit people who:

- Need to **see the point of their work the same day**
- Would rather be **moving than at a desk**
- Get *calmer* when things go wrong, not more scattered
- Take **variety over predictability**

It's harder if you need control over your own schedule, need quiet to function, or really dislike conflict — nurses have to push back on doctors, families and systems on a patient's behalf.

Being squeamish, for the record, is almost never the problem people expect.

If you change your mind

You're not stuck. Nursing opens onto advanced practice, specialisation, informatics, education, management, public health and industry. Few degrees have this many exits.

One catch, and it's the same pattern as everything else here: **the further a job sits from the bedside, the higher its exposure climbs.** Those are all good careers. Just move deliberately rather than by drift.

What actually matters when picking a programme

More than the ranking:

- **How many clinical hours, across how many different settings**
 - Whether they teach you to use AI tools *and* spot when they're wrong
 - Whether they build **assessment and judgment**, or just teach you to follow the protocol
 - What proportion of students finish — and where graduates actually end up
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Two things worth doing

Find a nurse and ask them properly. Not a family friend who'll be encouraging — someone who'll be honest about the bad parts. Ask: *what are the new nurses on your unit doing, and how is that different from when you started?* Half an hour with someone straight beats a term of careers advice.

Then ask yourself one question. Do you want *nursing*, or do you want what nursing is made of?

People are drawn to it for different reasons — helping people, the science and the body, high-pressure problem-solving, wanting a job that obviously matters. Those point in genuinely different directions, and it's worth knowing which one is yours.

If you want to go further — three things worth twenty minutes

- **The best and worst nursing specialties 2026** — read this one first. Satisfaction runs from about 68% down to 23% *inside the same profession*. It's really about which version of the job you'd want.
 - **2026 State of Nursing Survey** — over 2,000 nurses on pay, stress, safety and whether they'd do it again. Not a recruitment brochure.
 - **Ambient AI's next frontier may be nursing workflow** — how these tools are actually being used, and why nursing is harder for them than most people assume.
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There's a longer version behind this — full scores, how they were worked out, the honest downsides, what to ask an admissions office, and where we might be wrong. Your parents have it.